

IMAGES IN CLINICAL MEDICINE

Sézary Syndrome



A 27-YEAR-OLD MAN PRESENTED TO THE DERMATOLOGY CLINIC WITH A 6-month history of an itchy rash. Physical examination was notable for confluent, well-demarcated, hyperpigmented plaques across the torso and upper arms (Panel A, arrows point to areas of unaffected skin) resulting from the coalescence of small monomorphic papules (Panel B). Axillary and inguinal lymphadenopathy was also present. A skin-biopsy specimen showed a dense, perivascular, and pilotropic superficial dermal infiltrate of small T cells (Panel C), which were positive for CD2, CD3, CD4, MUM1, and PD1 and negative for CD7 and CD8. A peripheral-blood smear showed enlarged, atypical lymphocytes with convoluted nuclei, features that are consistent with Sézary cells. Immunophenotyping of peripheral blood and assays of T-cell clonality in blood and skin samples identified the same clone. A lymph-node biopsy revealed dermatopathic lymphadenopathy with interfollicular atypical T cells. Testing for human T-lymphotropic virus type 1 was negative. A diagnosis of the Sézary syndrome (stage B2) was made. The Sézary syndrome is an aggressive cutaneous T-cell lymphoma with leukemic involvement by malignant CD4+ T cells. The syndrome typically affects persons in late middle age and is often associated with erythroderma, unlike in this case. Treatment with systemic therapy was initiated, and a complete response occurred within 4 months. The patient had a relapse 1 year later and is currently undergoing evaluation for bone marrow transplantation.

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